



It's Okay To Not Be Okay

Mental Health Resources

- There are resources for every situation and every level of stress. There are many helpful resources available at UTSC's Health and Wellness Centre <https://www.utsc.utoronto.ca/hwc/>.
- An important part of the University experience is learning how and when to ask for help. Please take the time to inform yourself of available resources.
- Community Crisis Response Coordinator: Khadija Uddin: k.uddin@utoronto.ca
- Campus Police: **(416) 978-2222**
- Off-Campus: Good2Talk - a post-secondary (24/7) helpline (1-866-925-5454).
- If you or someone you know is in crisis:
 - If you're in immediate danger or need urgent medical support, call **9-1-1**.
 - If you or someone you know is thinking about suicide, call or text **9-8-8**. Support is available 24 hours a day, 7 days a week.



Psychological Disorders 3

Major Classifications of Psychological Disorders: Additional DSM Disorders

Lecture 18

Summer 2026

Dr. Anna Michelle McPhee

- By the end of class, students should be able to...
 - ...identify the characteristics of **Obsessive-Compulsive and Related Disorders**, and to examine specific types (**OCD and Hoarding Disorder**).
 - ...identify the characteristics of **Trauma- and Stressor-Related Disorders**, and to examine a specific type (**Posttraumatic Stress Disorder**).
 - ...identify the characteristics of **Substance-Related and Addictive Disorders**, and to examine a specific type (**Gambling Disorder**).
 - ...identify the characteristics of **Neurocognitive Disorders**, and to examine a specific type (**Alzheimer's Disease**).
 - ...identify the characteristics of **Personality Disorders**, and to examine a specific type (**Borderline Personality Disorder**).
 - ...explain the **Biopsychosocial Model**.

Daily Objectives for Today

- Obsessive-compulsive disorders are characterized by obsessions and compulsions.
 - E.g., OCD, Body Dysmorphic Disorder, Excoriation, Hoarding Disorder, etc.
- **Obsessions** are recurrent, unwanted and intrusive thoughts, fears, urges, or images.
 - E.g., Fear of germs/contamination.
- **Compulsions** are behaviours in response to an obsession in an attempt to reduce associated anxiety.
 - E.g., Repetitive handwashing.

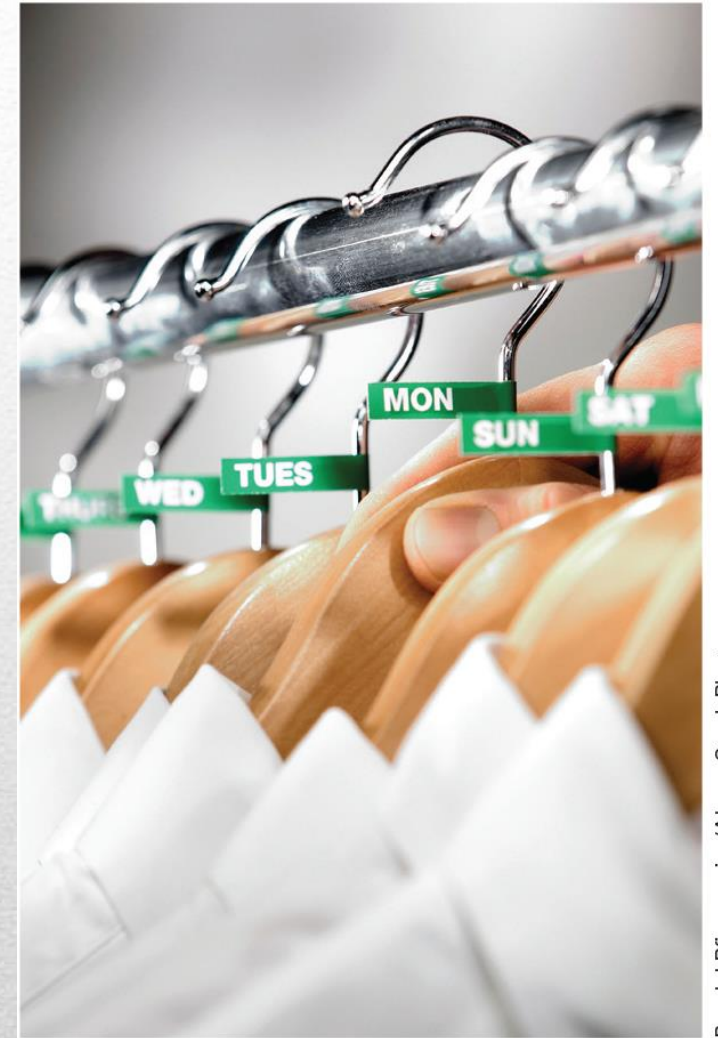


If an individual has an obsessive fear of germs, they may engage in compulsive behaviours such as repetitive hand-washing to try to reduce associated anxiety.

Obsessive-Compulsive and Related Disorders

Obsessive Compulsive Disorder (OCD)

- **OCD** used to be categorized as an anxiety disorder, but now is grouped as an obsessive-compulsive and related disorder.
- Risk Factors:
 - Twin studies suggest a strong genetic vulnerability for the disorder.
 - Symptoms of OCD may arise following head trauma, the brain inflammation caused by encephalitis and seizure disorder.
 - Young children who experienced birth complications or strep throat as children may be more likely to develop it later in life.
 - Lower levels of serotonin.



Randal Pfizenmaier/Alamy Stock Photo

Obsessive-Compulsive and Related Disorders



Individuals who have a hoarding disorder have difficulty getting rid of useless possessions, even if the items have no financial or practical value.

Hoarding Disorder

- Difficulty getting rid of useless possessions.
- Results in excessive accumulation of items.
- Areas in living space become unusable and hazardous (e.g., due to sanitation concerns, fire hazards, tripping hazards, health code violations, etc.).
- Can also have social consequences as well (e.g., isolation, conflict, loneliness, etc.).

Obsessive-Compulsive and Related Disorders

- **Trauma** is an emotional response to something shocking, terrifying, dangerous or life-threatening (e.g., car accident, natural disaster, assault).

- **Stress** is a reaction to life demands.

Posttraumatic Stress Disorder (PTSD)

- Experiencing or witnessing a traumatic event.
 - e.g., War, natural disaster, and/or violent crime.



Emergency medical staff regularly encounter trauma.

Trauma- and Stressor-Related Disorders

Posttraumatic Stress Disorder (PTSD)

- Symptoms of PTSD usually set in within 3 months of exposure to a traumatic event.
- Prevalence: 3.5% of the population experience symptoms of PTSD at any given time, and is projected that almost 9% will experience PTSD in their lifetime.
- Diagnosis: An individual must exhibit symptoms of 4 different categories to be diagnosed:
 - Re-experiencing symptoms.
 - Avoidance symptoms.
 - Cognitive and mood symptoms.
 - Arousal and reactivity symptoms.



Asahi Shimbun/Epa/Corbis Wire/Corbis

Trauma- and Stressor-Related Disorders

General Substance-Related and Addictive Disorders

- A class of psychological disorders related to behaviours that stimulate the brain's reward system to release dopamine and cause addiction.
 - Can be related to substances (e.g., drugs) or behaviours (e.g., gambling).
- **Addiction** is repeated and compulsive engagement in rewarding activities.
 - Disregard for negative, long-term outcomes.

- Substance-related Disorders can be categorized as:
 - **Substance Use Disorders**
 - **Substance Induced Disorders**
- All substance-related disorders lead to **physiological dependence** and **tolerance**.



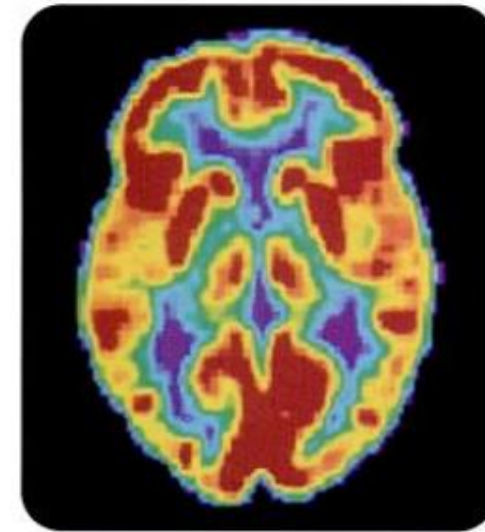
Compulsive gambling is considered a behavioural addiction.

Gambling Disorder

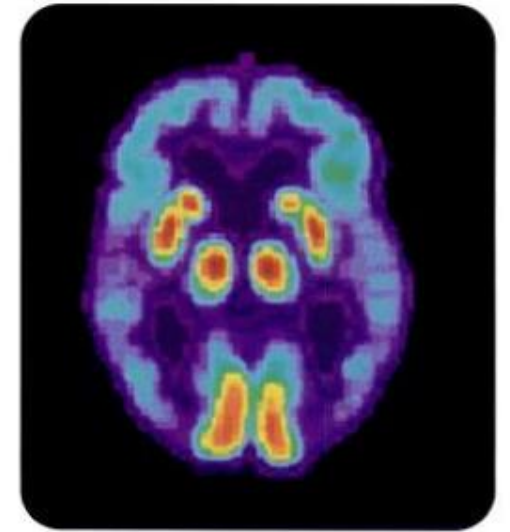
- Repeatedly placing bets to risk some money and gain even more.
- Chasing a feeling of euphoria.
- Considered a behavioural addiction.
- **Compulsive gambling:** an uncontrollable urge to continue gambling despite the consequences.
- Prevalence: 0.3%
- More commonly diagnosed in men.

General Neurocognitive Disorders

- Disorders related to problems with how the brain processes information due to underlying neurological conditions.
- Neurocognitive disorders are acquired.
 - Cognitive deficit was not present at birth.
 - E.g., delirium, major and mild neurocognitive disorders, Alzheimer's Disease
- Attributable to:
 - Brain Injury
 - Disease
 - Substance/Medication Use



PET Scan of Normal Brain



PET Scan of Alzheimer's Disease Brain

Alzheimer's Disease with significant structural changes in the brain.

Neurocognitive Disorders

General Personality Disorders

- Pervasive and enduring pattern of characteristics/traits, beliefs and behaviours due to one's personality that cause distress because they are drastically different from society expectations.
- First usually seen during adolescence or early adulthood; doesn't change over time.
- Arranged into 3 categories of behaviours:
 - 1. Cluster A:** Odd and Eccentric Behaviours
 - 2. Cluster B:** Dramatic, Emotional, and Erratic Behaviours
 - 3. Cluster C:** Anxious and Fearful

Personality Disorders

Borderline Personality Disorder

- Instability in interpersonal relationships, self-image, and emotion.
- Intense emotions and moods that quickly change.
- Thinking in absolutes of all good or bad/ black-and-white thinking.
- Individuals may try to self-soothe by engaging in impulsive, harmful behaviours (e.g., substance use, risky sexual encounters, excessive spending, self-injury and suicidal behaviours).
- May engage in frequent and quick life changes that often occur without warning, advance preparation or consideration of consequences.

Personality Disorders: Cluster B

Personality Disorders: Borderline Personality Disorder

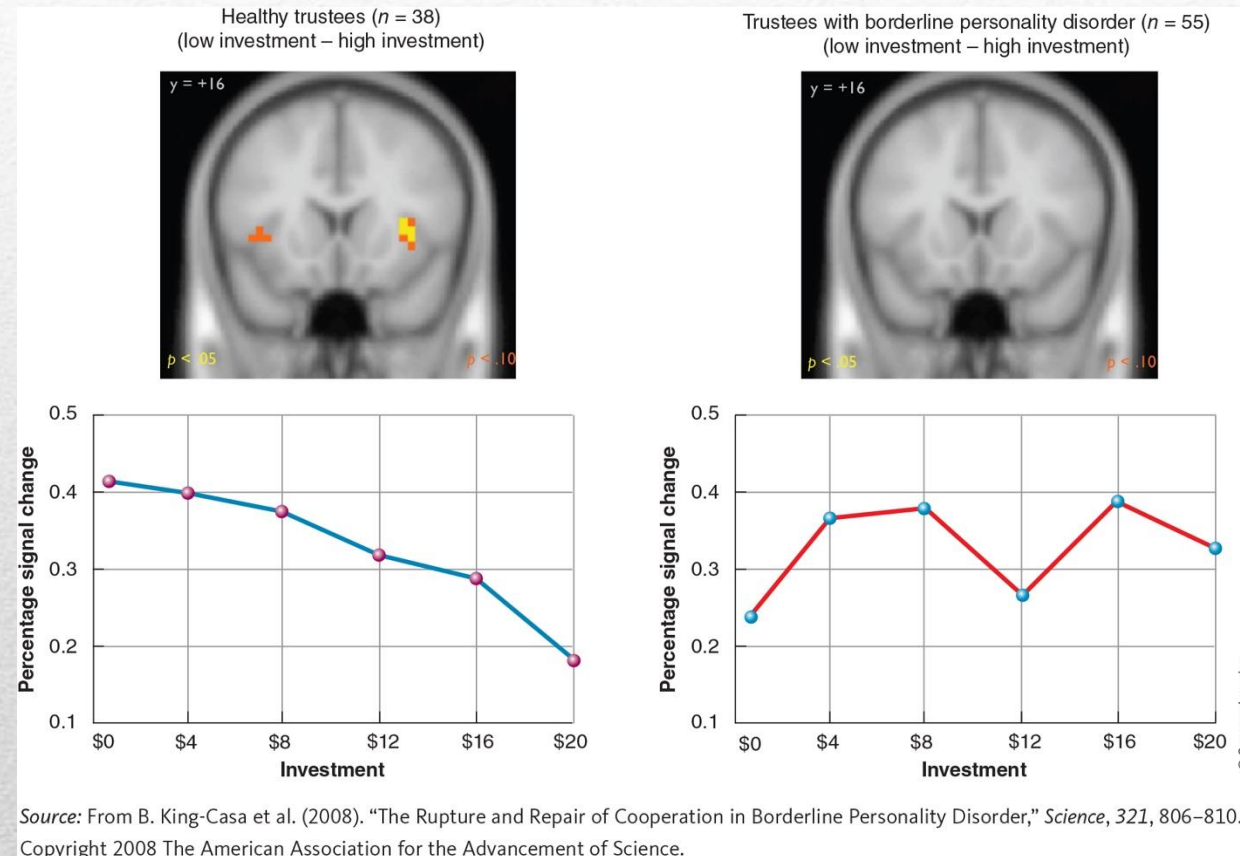
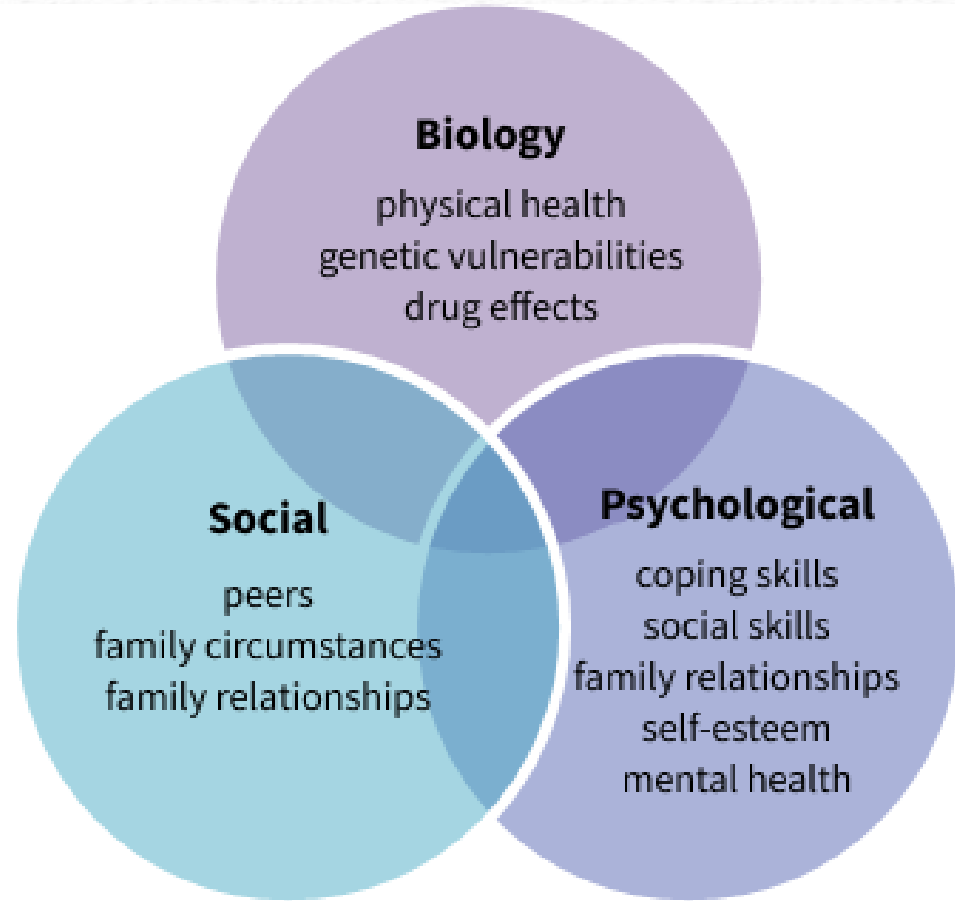


Figure 14.20 Abnormal Responses to Trust in Borderline Personality Disorder

The Biopsychosocial Model

- Many factors and events cause a psychological disorder.
- Interactions determine risk.
 - Biological Makeup
 - Psychological Experiences
 - Social Environment



The Biopsychosocial Model

What Causes Psychological Disorders?

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